

**CampusTown Urgent Care: Understanding Student Health Insurance Literacy in the  
Context of National Policy**

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After a semester spent interning at CampusTown Urgent Care on the University of Illinois Urbana-Champaign campus, I encountered health insurance concerns unique to the college student population. The U.S.'s fluctuating financial state and administration changes have made it hard to anticipate consistent support by government services like Medicaid, making university-offered health insurance plans even more important for students. Taking advantage of the available benefits can make a big difference in respective academic success and well-informed post-grad planning. The purpose of this report is to address challenges faced in making health insurance decisions, discuss relevance to University of Illinois students, and propose intervention for health insurance literacy.

## **I. Background**

U.S. Health Insurance as we know it came about during the Great Depression in the 1930's. Spearheaded by Baylor University Hospital in 1929, a "prepaid plan" covered 1,300 Dallas school teachers during a period of great financial hardship, allowing each to pay 50 cents a month in exchange for 21 days of medical care in their facility. The success of this innovative payment option led to the development of "Blue Cross plans" by the American Hospital Association, essentially providing coverage for care at all hospitals in the community. The "Blue Shield plans" that followed covered doctor's visits and services provided by physicians, organized by employers of loggers and miners who needed their employees to stay strong to keep the business productive (Blue Cross Blue Shield, 2024). Those efforts became what we know as Blue Cross Blue Shield, a leader in US health insurance made strong by that Baylor plan. This historical time of instability created a principle of healthcare that we are still following today, which continues to shape initiatives of the federal government, helping provide healthcare coverage for all Americans.

In 1945, President Truman was the first president to propose the concept of national health insurance to Congress. 20 years later President Johnson signed the establishing legislation of Medicare and Medicaid in 1965, increasing access to health insurance for older adults and certain low-income groups (Hoffman & Kaiser Family Foundation, 2009). Ongoing reforms by different administrations to program eligibility and benefits have worked to improve Americans' access to medical care. In 2010, the Obama Administration introduced the Affordable Care Act

(ACA), an expansion of healthcare so notable that it led to a significant drop in the rate of uninsured Americans. One of the first regulations of the act allowed young adults to remain on a parent's health insurance plan until age 26, providing insurance protection in a transitional and arguably turbulent period of life. This especially changed the coverage landscape for college students, ensuring continuous coverage for those still focusing on their education. ACA also launched the Health Insurance Marketplace, allowing people to shop and access affordable coverage options that suit their needs. It also raised many standards for student health insurance plans, most of which are regulated by the ACA (Mishory, Chan, & Granville, 2020). Colman & Dave (2016) note that this new coverage provision resulted in increased time college students spent on "educational activities". Thus, having confidence in health insurance coverage has the potential to encourage young adults to pursue degrees in higher education, relieving stress from financial strain throughout their lives.

## **II. National Context**

Though many benefits have come from national health insurance efforts, the U.S.'s fluctuating financial state and changes in administration creates an environment for disagreement and slow reform to healthcare insurance policy. One recent challenge is the vulnerability of Medicaid to federal budget cuts, which has been a high-pressure area of focus under the current administration. The recent "One Big Beautiful Bill Act" from the Trump Administration, signed into law July 4<sup>th</sup>, 2025, cuts federal funding for Medicaid by \$1 trillion over the course of 10 years (H.R.1, 2025). The Congressional Budget Office (2025) expects such changes in federal health insurance funding and financing to jeopardize affordable healthcare insurance access, causing 16 million Americans to lose Medicaid access. Factors like inflation and economic uncertainty can drive insurance premiums up, resulting in financial strain when trying to access necessary healthcare services (Cox, 2025). The Department of Education has also recently come under federal and press attention, as the White House has shared goals for more budget cuts and major department shifts. The Trump Administration (2025) called for a \$12 billion cut to a group of Department of Education programs, \$1 billion of which being a grant funding mental health professionals in schools. Mental health implications because of this change may source from large amounts of staff layoffs in both school mental health programs and the federal Substance

Abuse and Mental Health Services Administration, ultimately failing to reach young adults who need the resources.

Medicaid has long served as a primary tool to provide young adults with healthcare coverage. US Census Bureau data shows that young adults ages 19-34 have the highest uninsured rates compared to all age groups (Conway, 2020) and have increasingly utilized Medicaid since its expansion by the ACA. Young adults then transitioning to self-sufficiency post-graduation have not yet advanced their career, and entry-level work typically places them in low-wage jobs, subjecting them to continued financial uncertainty when weighing expensive health insurance options. The cuts to school mental health programs funded by the Department of Education create more concern for this age group, considering suicide is the second leading cause of death in ages 10-24 (National Institute of Mental Health, 2025), with suicidal ideation and behaviors being preventable with mental healthcare. Moreover, the costs of university-offered student health insurance plans may be a roadblock to healthcare. Rising plan costs can influence care-seeking behavior and even lead groups of low-income or financially independent students to go without coverage.

Interruptions in Medicaid or university-sponsored health insurance access could be detrimental to the health of young adults in particular, leaving many of them dangerously uninsured during a life period characterized by change and exploration. Student growth and success are vulnerably reliant on financial and policy stability. Recent budget cuts create concern not only for their health, but future endeavors and opportunity for their adult lives.

### **III. United Healthcare Student Insurance**

The University of Illinois requires campus-based students registered for non-zero credit hours to have health insurance, in order to offset the potential costs of healthcare during their time as an undergraduate. The University offers a comprehensive, major medical Student Health Insurance plan, charged to all Illinois students in their tuition. At a cost of \$846 per semester, the Student Health Insurance plan includes coverage on inpatient, outpatient, emergency, preventative, and mental health care, along with coverage on durable medical equipment and prescription drugs. The plan has “Gold” status in the ACA Marketplace, assigning a higher

premium or regular payment, but lower out-of-pocket costs, with a variety of benefits for the student. It has a \$400 deductible, which is the amount a student would pay for covered medical services before the Student Health Insurance plan starts paying. After the deductible is met, the plan provides an 80% coinsurance on covered medical services, where the student would only need to pay the remaining 20% of the balance (UnitedHealthcare, 2025). The coverage is accessible on campus, at home, and even while traveling the world, providing a thorough health insurance plan for all students.

A student's coverage is therefore determined by their decision to opt-out or fully enroll in the University plan. If a student is covered under another comparable plan, for example a parent's health insurance provided by their employer, they will need to successfully opt-out/waive the Student Health Insurance during the fall enrollment period of each academic year. This waiver is open during the first six weeks of the fall semester, and is subject to university approval based on four requirements: (a) is a US-based plan, (b) coverage is effective on/before the Saturday before the first day of classes for the given semester and extend through the academic year, (c) provides access to in-network emergent and non-emergent care within a 50-mile radius of the UIUC campus, and (d) is compliant with the Affordable Care Act. An approved waiver will apply for only the assigned academic year, requiring a new waiver each year. This opt-out waiver will credit \$846 off tuition each semester. Alternatively, if a student decides to accept the Student Health Insurance plan, they will be automatically opted-in but still need to complete the Enrollment Form. This step ensures complete enrollment to the plan, allowing access to their digital insurance card and opportunity to add dependents. The important dates table found on the [Student Health Insurance website](#) is a helpful resource for students to plan for the enrollment/waiver periods each academic year.

#### **IV. Clinic Observations**

As a college senior, I found my time working at a campus urgent care this fall to be thought provoking, as to what knowledge I and the rest of the undergraduate population at my university have regarding healthcare insurance. An average day working at CampusTown Urgent Care involved a multitude of clinical administrative tasks, navigating encounters with patients of varying needs, and working towards highly efficient billing operations each day. My experience

seeing the general knowledge of health insurance among undergraduates first-hand was concerning. Students would hand over a copy of their parent's insurance card, not understanding their coverage, possible copays, and critical policy details like whether a prescription drug would be covered. A common misunderstanding, encountered almost daily, was the assumption that "coverage" directly meant "full" coverage. I often shared with patients that this is not always the case and depends on the policy details of the insurance plan. When dealing with patients insured by plans out-of-network, sharing that information with the patient was difficult, as they displayed a lack of understanding of insurance networks at all. Most patients shared with confidence what insurance provider they carried, very often being a dependent under a parent's employer-sponsored plan. Some students with Medicaid plans expressed a lack of confidence in their coverage, indicating national news about ACA and Medicaid left them feeling uncertain. Many students were largely unaware of the Student Health Insurance plan offered by the university as well, sometimes sharing they had "heard of it before", but not being aware of any action taken. Even those who had the UnitedHealthcare Student plan most often did not have a copy of their insurance card, as they did not know where to access it. The differentiation between the automatic opting-in and complete enrollment was unknown to nearly all encountered patients, where we would then require the student to complete the enrollment steps in-office, in order for me to enter the insurance information for verification, before being seen by a doctor. This commonly led to backups in the waiting area and created an unnecessary barrier to their care.

For students, the most common intended use of health insurance is for primary care (Wong et al., 2015), but urgent care showed a far different variation in utilization. Comparatively, primary care visits are typically non-urgent and scheduled, where an urgent care visit is more of a display of intentional healthcare utilization. In my time at CampusTown Urgent Care, it became clear to me that a college student will seek the care they believe they need, only upon deciding it is necessary, and less often with consideration of their insurance status or what it means for them financially. Still, I noticed that national news about Medicaid interruptions created a reasonable amount of discomfort in many students I encountered, reflecting their uncertainty about the stability of such plans. I suspect other population of Americans utilizing Medicaid are experiencing similar worries. I am concerned this is significant enough to risk effective utilization of these healthcare services, as well as overall health outcomes of both students and non-student populations utilizing Medicaid.

## **V. Insurance Literacy**

An increasing amount of research has been done on health insurance literacy in the last 10 years, with a handful of papers having a directed focus on young adult and college student populations. One study found that a sample of young adults had 48% of participants incorrectly define “deductible” and 78% incorrectly define “coinsurance” (Wong et al., 2015). The sample of highly educated young adults were confused by common insurance terms, so they also found the process of selecting and enrolling in a health plan confusing and challenging. Research of this kind provided valuable insight into the young adult’s perspective on health insurance and even led to new communication and engagement strategies for the young adult population’s health insurance matters. Nobles et al. (2018) indicated that although college students are considered a highly numerate and literate population, higher education may not correlate with increased ability to navigate the U.S. healthcare system. The study found that approximately one-quarter of college student participants indicated that their lack of understanding stopped or delayed them from seeking medical care in the past. This is both a concern and an opportunity, for universities to set their students up with a reliable health insurance plan and proper education on navigating the system later in life. Horner et al. (2022) assessed university student’s health insurance acquisition preparedness, with findings indicating high levels of stress regarding health insurance conversations with future employers’ post-graduation. This accurately displays the climate college students find themselves in while future-planning.

Early health insurance literacy education is key for future preparedness. Low health insurance literacy in Americans has been estimated to be a cost of \$106-\$238 billion to the U.S. economy (Mahoney, 2022). Upadhyay et al. (2022) found that students enrolled in a health insurance course scored far higher on a measure of health insurance knowledge than their counterparts without that formal education. Education in undergrad can be a large contribution to the preparedness of students for their post-graduate lives, a time where health insurance decisions will increase. The exposure of insurance concepts and healthcare utilization methods can effectively prepare college students to seek insurance plans and healthcare as they go through their young adult life. Finally, the success of large health insurance plans is reliant on the enrollment of younger, healthier individuals to balance/offset the costs of other individuals on the same plan who may require more health services (Nobles et al., 2018).

## **VI. Proposed Intervention**

Many institutions have established health insurance courses and even carried out interventions to address the knowledge gap observed among college students. One notable example comes from a collaboration of the University of North Texas Health Science Center and the University of Arkansas Medical Sciences Center, who created a health insurance guided course for students, breaking down the complexities of health insurance and setting college students up for a smooth transition off their parent's insurance plans. The course showed to be successful and later received an award as a comprehensive digital health strategy (UNT Health, 2025). The [University of Illinois Student Health Insurance](#) has healthcare insurance education resources available, answering frequently asked questions about insurance terms, enrollment, and student plan details. There are gaps seen in this method though, confirmed by my own first-hand experience at CampusTown Urgent Care. I have considered that if students are not aware of where they can access their digital insurance card, we cannot expect them to understand to have a strong understanding of the health insurance they pay for.

My strategy for improving health insurance literacy among undergrads at the University of Illinois is to provide a low health insurance-literacy resource as a call-to-action. I designed a policy-informed infographic to reach students accessing urgent care at the clinic, answering questions about what the student plan entails, why students should pay attention to health insurance, and even included QR codes, directing students to opt-out and enrollment directions. The intended use of my infographic is for the urgent care setting, but I believe can be useful for any clinical or educational setting. This resource may also be shared with the University, reaching more students if displayed in university-run campus health center McKinley Health Center, the Illini Student Union, and other university buildings like lecture halls. My educational infographic can be taken further by the greater institution, emphasizing the importance of the health insurance plan they are automatically eligible for and understanding the health insurance system as a whole, setting them up for future success.

## **VII. Conclusion**

This report attempts to contextualize health insurance information based on national history, institution-wide efforts, and individual student experience. I found health insurance literacy relevant in my experience as an intern at CampusTown Urgent Care, this being my inspiration to cover the need for more discussion among my peers. The self-designed infographic I created for the University of Illinois Student Health Insurance plan is among the efforts needed in campus healthcare settings. More research on health insurance literacy and new education methods are needed in universities. Initiatives must meet students where they are, engage their attention to insurance concepts, and meet their unique needs as a transitional group of young adults.

## References

- Blue Cross Blue Shield. (2024). History of Health Care. Retrieved from Blue Cross Blue Shield Association website: <https://www.bcbs.com/about-us/blue-cross-history-of-healthcare>
- Colman, G., & Dave, D. (2016). It's About Time: Effects of the Affordable Care Act Dependent Coverage Mandate on Time Use. In *Ssrn.com*. Retrieved from <https://ssrn.com/abstract=2731973>
- Conway, D. (2020). Uninsured Rates Highest For Young Adults Aged 19 to 34. Retrieved from The United States Census Bureau website: <https://www.census.gov/library/stories/2020/10/uninsured-rates-highest-for-young-adults-aged-19-to-34.html>
- Cox, C. (2025, October 29). ACA Insurers Are Raising Premiums by an Estimated 26%, but Most Enrollees Could See Sharper Increases in What They Pay. Retrieved from KFF website: <https://www.kff.org/quick-take/aca-insurers-are-raising-premiums-by-an-estimated-26-but-most-enrollees-could-see-sharper-increases-in-what-they-pay/>
- Hoffman, C., & Kaiser Family Foundation. (2009). [Review of *National Health Insurance — A Brief History of Reform Efforts in the U.S.*]. *Kff.org*, 5. Retrieved from <https://www.kff.org/wp-content/uploads/2013/01/7871.pdf>
- Horner, A., Gregory, B., & Bembas, R. (2022). Assessing health insurance acquisition preparedness in university students. *Journal of American College Health*, 72(7), 1–8. <https://doi.org/10.1080/07448481.2022.2100998>
- Mahoney, M. E. (2022). Digital Solutions to Improve Health Insurance Literacy. *Journal of Medical Systems*, 46(8). <https://doi.org/10.1007/s10916-022-01844-w>
- Mishory, J., Chan, O., & Granville, P. (2020). The ACA's Impact on College Students. In *The Century Foundation*. Retrieved from <https://tcf.org/content/report/the-acas-impact-on-college-students/>
- National Institute of Mental Health. (2025, March). Suicide. Retrieved from National Institute of Mental Health website: <https://www.nimh.nih.gov/health/statistics/suicide>
- Nobles, A. L., Curtis, B. A., Ngo, D. A., Vardell, E., & Holstege, C. P. (2018). Health insurance literacy: A mixed methods study of college students. *Journal of American College Health*, 67(5), 469–478. <https://doi.org/10.1080/07448481.2018.1486844>
- One Big Beautiful Bill Act, H.R. 1, 119th Cong. (2025)

- Student Affairs. (2025a). FAQ Basics | Student Health Insurance. Retrieved December 8, 2025, from Illinois.edu website: <https://si.illinois.edu/faqs/faq-basics>
- The Congressional Budget Office. (2025, September 18). The Estimated Effects of Enacting Selected Health Coverage Policies on the Federal Budget and on the Number of People With Health Insurance. Retrieved from Congressional Budget Office website: <https://www.cbo.gov/publication/61734>
- Trump Administration. (2025). *Executive Office of the President | Office of Management and Budget* (pp. 4–9). Retrieved from <https://www.whitehouse.gov/wp-content/uploads/2025/05/Fiscal-Year-2026-Discretionary-Budget-Request.pdf>
- UnitedHealthcare. (2025). *University of Illinois Undergraduate Health Insurance Plan* (pp. 1–2). Retrieved from [https://si.illinois.edu/sites/default/files/2025-09/2025-1351-1%20Summary%20Flyer\\_updated%208.22.25.pdf](https://si.illinois.edu/sites/default/files/2025-09/2025-1351-1%20Summary%20Flyer_updated%208.22.25.pdf)
- UNT Health. (2025, February 28). Empowering young adults through health insurance literacy. Retrieved from Newsroom website: <https://www.unthsc.edu/newsroom/story/empowering-young-adults-through-health-insurance-literacy/>
- Upadhyay, S. S. N., Merrell, L. K., Temple, A., & Henry, D. S. (2022). Exploring the Impact of Instruction on College Students' Health Insurance Literacy. *Journal of Community Health, 47*. <https://doi.org/10.1007/s10900-022-01096-2>
- Wong, C. A., Asch, D. A., Vinoya, C. M., Ford, C. A., Baker, T., Town, R., & Merchant, R. M. (2015). Seeing Health Insurance and HealthCare.gov Through the Eyes of Young Adults. *Journal of Adolescent Health, 57*(2), 137–143. <https://doi.org/10.1016/j.jadohealth.2015.04.017>